

A.3.9

Policy/Guideline Title:	Medical Staff Leave Policy		
Executive Summary:	This policy governs leave entitlement and applies to all medical and dental staff employed by the Trust. Leave for doctors and dentists should be managed within a framework that allows medical staff to take their leave entitlement and for patient care to be delivered effectively and consistently throughout the year. A pre-planned and appropriately managed leave system is essential in providing an excellent service to patients.		
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A.3.9: Medical Leave Policy



Compassion



Accountability



Respect



Integrity



Courage

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1. Introduction

This policy governs leave entitlement and applies to all medical and dental staff employed by the Trust. Leave for doctors and dentists should be managed within a framework that allows medical staff to take their leave entitlement and for patient care to be delivered effectively and consistently throughout the year. A pre-planned and appropriately managed leave system is essential in providing an excellent service to patients.

2. Purpose

The policy aims to ensure that:

- All doctors take leave in accordance with service needs (these will normally be defined by the appropriate Clinical Director/Lead Consultant);
- Appropriate cover is maintained for service delivery at all times (as defined by the Clinical Director with advice from the appropriate Divisional Director or Associate Divisional Managers);
- Accurate electronic records of all leave periods are maintained;
- There is a process of continued monitoring, review and evaluation of leave based on service needs;
- Requests for leave are handled in a fair, transparent and consistent manner;
- All leave is appropriately authorised;
- Disruption of patient-centred duties (such as wards rounds, theatre sessions, clinics etc.) is minimised;
- Bank/Agency staff are only employed in exceptional circumstances.

These guidelines apply to all medical and dental staff with clinical duties at Kettering General Hospital NHS Foundation Trust, regardless of the number of hours worked or their actual employer. The entitlements given below apply to those who are employed by the Trust and will be used as guidelines for agreeing leave from Kettering for those employed by other Trusts.

3. Responsibilities

The responsibilities and accountabilities for monitoring, reporting and recording of leave unless otherwise clearly documented, communicated and agreed by the Clinical Directors are as follows:

Clinical Directors to:

- Ensure that electronic leave records are used and maintained. The Clinical Director may choose to delegate these responsibilities but they will ultimately have the final say on approval of leave. The Clinical Director will remain accountable for any decisions taken on their behalf.
- Normally authorise (but can delegate this responsibility to Associate Divisional Managers) leave requests for consultants, taking account of the relevant points of the guidelines and the needs of the service. Requests for leave should not normally be granted where this would be to the detriment of the service.

- Consultants must forward plan their leave requirements so as to be able to plan all direct clinical care. Disruption to services should be minimised as a result of a consultant's annual or study leave arrangements.
- Ensure that consultants take leave with a proportionate impact on direct care and supporting activity depending on their job plan. If it is noted that leave requests do not have a proportionate distribution between direct care activity and supporting activity then the Clinical Director may ask the consultant to alter their leave request or future leave requests accordingly.
- Allow consultants to take up to two days of their total annual leave allowance without needing to use the formal application process (as allowed in the Terms and Conditions of Service for Consultants), provided that they give notification beforehand and suitable cover arrangements can be made. The electronic system will need to be updated.
- Refer to the Chief of Division or Divisional Director any cases of noncompliance with reporting procedures or recurring incidents of unplanned absences.
- Monitor unplanned leave and ensure that all such leave is properly recorded.

All Medical and Dental staff to:

- Agree in advance with their Clinical Director or Clinical Lead (Clinical or Line Manager) any planned leave that they wish to take and to maintain electronic leave records.
- Notify any unplanned absence as soon as possible on the first day and confirm the reason and expected date of return.
- Give as much notice as possible of intended absence due to annual/study leave with a minimum of six weeks' notice in any case. Ideally leave requests would be made seven weeks ahead to ensure adequate time for approval. The impact of leave on services will vary according to the time of year, size of clinical team and amount of leave requested. Staff should forward plan their leave requirements so as to be able to deliver all activities in their job plan. Disruption to services should be minimised.
- Note that requests to take a significant proportion of leave as individual days throughout the year impacts on the ability of others to take their leave. This is especially the case in small teams / specialties.
- Be encouraged to take a proportion of their leave in week blocks. If the pattern of leave requested by any individual member of a team restricts the ability of another member of the team to take their leave then the authorising manager will be expected to balance the needs for all staff and agree or refuse the request for leave accordingly. Leave requested for over two weeks should be authorized by the Chief of Division or Divisional Director before being approved.

Medical Director to:

- Ensure overall compliance with this policy.

4. Definitions

Consultant - Doctors being paid on the permanent or locum consultant pay scales. A permanent consultant will be on the GMC specialist register.

Associate Specialist, Specialist, Specialty Doctor - Experienced doctors in permanent posts. They have at least four years postgraduate training, two of which in the relevant specialty to enter the Specialty Doctor grade.

Trust Grade - The broad terms used for Trust appointed doctors whose terms and conditions mirror those in training grades.

Training Grade doctor - Doctors on approved training programmes such as Foundation Year (FY1, FY2), core training (CT1, CT2), Specialty Training (ST1 to ST7).

Senior Medical Staff - Generally Consultants and SAS doctors.

Junior Medical Staff - Doctors in the Trust Grade or Training Grade groups.

5. Annual Leave Entitlements and Authorisation

Annual leave should be taken in line with national guidelines. The entitlements below are based on a five day working week, adjustments will be made for those who work less than full time or full time but less than 5 days.

Grade	Entitlements	
Consultant	Full-time consultants with up to 7 years' completed consultant service are entitled to 32 days' annual leave, which includes two statutory days converted to annual leave. Full-time consultants who have completed 7 years' plus at consultant level are entitled to 34 days' annual leave which includes two statutory days converted to annual leave. Consultants appointed at KGH prior to April 1997, based on the policy at that time, are eligible to three statutory days, instead of two converted to annual leave.	8 public holidays per year are also allowed (but this may vary slightly year on year and entitlements will be adjusted). Medical staff who in the course of their duties are required to be present in hospital or other place of work on statutory or public holidays should receive a day off in lieu.
Associate Specialist, Specialty	32 days per year pro rata	
Trust Grade SpR, ST3 to ST8	32 days per year pro rata if have 5 or more years NHS service or equivalent. 27 days per year if have less than 5 years NHS service or equivalent.	

Trust Grade SHO, ST1s and ST2s, CT1s and CT2s	32 days per year pro rata if have 5 or more years NHS service or equivalent. 27 days per year if have less than 5 years NHS service or equivalent.	
FY1/FY2	27 days per year pro rata 9 days in each 4-month period	

Pro-rata adjustments (see below) apply to any doctor contracted for fewer than 10 PAs/40 hours per week or those that work less than 5 days per week. Note, these pro rata adjustments apply irrespective of how many hours are worked per day.

Days worked in a Normal week	Entitlement – 34 days Per annum	Entitlement – 32 days Per annum	Entitlement – 27 days Per annum
5 days normally worked	34 working days	32 working days	27 working days
4 days normally worked	27 working days	25.5 working days	21.5 working days
3 days normally worked	20.5 working days	19 working days	16 working days
2 days normally worked	13.5 working days	13 working days	11 working days
1 day normally worked	7 working days	6.5 working days	5.5 working days

Authorisation Process for Consultant/Associate Specialist/Specialty Doctors

Clinical Directors are responsible for agreeing leave for consultants within their Directorate and Chief of Divisions for the Divisional unit as a whole (in their absence the Divisional Director will be able to authorise leave). In doing so, service needs must be taken into account, and should only be granted where adequate cover is provided by consultant colleagues within the department.

The Clinical Leads/Clinical Director/Chief of Division and Divisional Managers should agree on how many consultants/senior staff can be away at any given time.

Consultant/Associate Specialist/Specialty Doctors wishing to take leave must complete the appropriate electronic request (see Appendix A) and where appropriate /necessary, in the notes section of the electronic form, specify the Consultant/Associate Specialist/Specialty Doctor colleague who would cover necessary duties in their absence. Consultants with on-call commitments must ensure that adequate cover is agreed with colleagues prior to submitting the request. Requests for leave must be submitted for approval no later than 6 weeks prior to the intended leave date or the leave may not be authorised.

It is the responsibility of the Clinical Director/Chief of Division to ensure that the department is adequately covered before signing off a leave request.

Annual Leave Year for Senior Medical Staff

The leave year runs from 1 April to 31 March.

Carry Over of Annual Leave for Senior Medical Staff

The Trust position regarding carryover of consultant annual leave is, on the whole consistent with non-consultant staff in that annual leave should only be allowed to be carried over in exceptional circumstances. Medical Staff, therefore, are encouraged to take annual leave in full within each leave year and leave should be planned and taken proportionally through the year to promote health and well-being.

However, based on the contractual difference for Consultant staff, an individual may carry forward up to 5 days annual leave, outside of an exceptional circumstance and subject to the exigencies of the service. In these instances, the Chief of Division and Divisional Director should be notified.

No payment will be made for any untaken leave within a financial year. To ensure the Trust complies with its financial accounting policies, annual leave data, including the number of days of untaken leave at 31st March must be made available by the Division to the finance department at the end of each financial year.

Record Keeping for Leave

Adequate and up-to-date electronic records must be maintained by Divisions on consultants in each department who are on leave at any given time. Each Division will agree the staff job titles responsible for maintaining leave records and advise the appropriate Rota Co-ordinator.

Authorisation Process for Junior Medical Staff Leave

Consultant supervisors are responsible for agreeing leave for doctors within their Department. In doing so, service needs must be considered and leave should only be granted where adequate cover is provided by colleagues within the department. This role may be delegated to ADM or Rota Coordinator as long as clear rules have been identified for staffing levels.

Doctors wishing to take annual leave must complete the appropriate electronic leave request (see **Appendix A**) and specify, in the notes section of the electronic form, colleagues who will cover necessary duties in their absence. Requests for leave must be submitted to the Consultant no later than 6 weeks prior to the intended leave date or the leave may not be authorised.

It is the responsibility of the Consultant to ensure that the department is adequately covered before signing off a leave request.

Without an authorization any leave request will be deemed invalid and the doctor will be expected to work.

Annual Leave Year for Junior Medical Staff

For SpR and ST3-ST8, the leave year runs from the start date of a Doctor's employment with the Trust. For Trust Grade SHO, ST1s and ST2s, CT1s and CT2s FY1 and FY2 all leave must be taken within each 4/6-month period with no carry over.

Carry Over of Annual Leave for Junior Medical Staff

Annual leave should be taken in full within each leave year. However, a maximum of 5 days' leave may be carried over from one leave year to the next in exceptional circumstances only. Any leave carried over must be agreed with the Clinical Director and Divisional Director before the end of the leave year. To ensure the Trust complies with its financial accounting policies, annual leave data, including the number of days of untaken leave at 31st March must be made available by the Division to the finance department at the end of each financial year.

6. Study and Professional Leave

Consultant/Associate Specialist/Specialty Doctor/Trust Doctors

Background

The Trust recognises the need for all Senior Medical Staff to maintain good medical practice through continuing medical education and professional development to deliver excellence in healthcare. The Trust also recognises the significant impact General Medical Council (GMC) revalidation will have on the uptake and time allocated for professional and study leave.

Study Leave

Study leave can be defined as 'a period of leave from the workplace taken for the purpose of enhancing professional knowledge and skills. This includes study or training, usually but not exclusively or necessarily on a course or programme. Examples include attending, presenting or chairing a conference session; research or studying for professional qualifications; taking examinations. It is important to recognise that Consultants are required in relation to the terms and conditions of

employment to keep up to date with the elements of Mandatory Training identified in the Trusts Training Needs Analysis.

Professional Leave

This may be defined as ‘a period of leave from the workplace, taken for the purpose of using professional skills and experience for the wider benefit of the NHS, health and social care nationally or internationally.’ This includes committee and other work for recognised professional bodies and specialist associations (e.g. Royal Colleges, GMC, CQC); teaching and training (lecturing outside of the region); provision of expert professional advice to Courts of Law; taking part in visiting clinics; acting as an external examiner. Where leave is for Trade Union duties please refer to Time off for Trade Union duties and activities Procedure.

Working Off-Site

This applies where a Kettering Consultant visits another centre to advise or gain knowledge of procedures and clinical activity. It also includes lecturing to undergraduates and post graduates within the region. Working off site does not include providing a service to patients at clinics held on other NHS premises. There is no leave limit to the amount of off-site working but individuals need to agree with the Clinical/Medical Director a balance between service need and benefit to the Trust and wider NHS.

Sabbatical and Secondment

The Trust may grant a sabbatical/Career/Employment Break or secondment and further information can be obtained in The Trust Employment Break Policy. A sabbatical/Career Break/Employment Break is a period of extended leave, usually unpaid, to allow an employee to develop their career or pursue a topic of special interest. A secondment is usually a temporary placement for a specific purpose and period of time to the mutual benefit of all parties. Any proposal for leave should be made before the annual appraisal and considered in the annual Job Plan review. Approval will only be given subject to the needs of the service and providing that enough notice is given. In most cases sabbaticals can be covered by additional professional leave. All leave must be authorised by the appropriate Clinical Director, Chief of Division/ Divisional Director and Medical Director (Decision Making Group). The application for Medical Staff is included in **Appendix C**.

Recommended Allowance

Schedule 18, paragraph 13 of Terms and Conditions for Consultants identifies that *“professional or study leave will normally be granted to the maximum extent consistent with maintaining essential services in accordance with the recommended standards”*. The recommended standard for consultants is leave with pay and expenses within a maximum of thirty days (including off-duty days falling within the period of leave) in any period of three years for professional purposes. There is no separate allocation of professional leave.

Location of Study/Professional Leave

Study/professional leave will normally be undertaken in the EU however with approval, leave can be taken outside the EU.

Booking of Leave

Study/professional leave must be requested from the appropriate Clinical Director or Chief of Division or Divisional Director in the case of Clinical Directors, prior to the leave being taken and with a minimum of 6 weeks' notice. Retrospective applications for leave are not allowed and expenses will not be paid.

All leave, including study, professional leave and working off-site should be requested using the Form found in **Appendix B**. Internal education and development courses should be considered in the same way as external study/professional leave.

Clinical Directors will normally grant leave up to the maximum extent consistent with maintaining essential services.

Study leave should reflect the personal development needs identified during the annual appraisal and be relevant to the Consultant Job Plan. The potential benefit to the Trust or the wider NHS of professional leave should be discussed.

Study/Professional leave more than the locally agreed allocation can only be granted by the Chief of Division or Divisional Director in consultation with the relevant Clinical Director and must be agreed by the Medical Director.

For study leave the costs of travel, accommodation and course/conference fees will normally be funded, up to the maximum amount allowed by the Trust, reviewed annually. The rules regarding these expenses are below.

Only the duration of the actual study/professional activity plus reasonable travel time should be taken, e.g., travel the same day if it is reasonable to do so.

Where study/professional leave with pay is granted, staff must not undertake any paid work for another body. Where paid work is undertaken for another body this must be taken as annual leave unless fees are paid to the Trust. Lecture fees and minimal honoraria undertaken in Job Plan time may be retained to cover the costs of travel and preparation time, with the agreement of the Trust. Where an individual is attending Court as an Expert Witness and this is not on behalf of the Trust, annual leave must be requested.

Individuals may not normally claim expenses from the Trust for participation in professional leave activities and expenses should be claimed from the relevant external body. Only when expenses cannot be claimed from another source should expenses be requested from the Trust. Educational Tutors/programme leaders responsible for the delivery of under or postgraduate teaching of doctors at the Trust may claim expenses for attendance at essential meetings at Colleges, HEE or Medical School.

Where a practitioner is employed by the Trust and another employer, it is the responsibility of the Trust holding the contract of employment to plan for the approval and funding of study/professional leave and the purposes for which it is required must be agreed at the point that the employment arrangements are determined.

If an individual is considering taking up a significant external role (e.g. Regular working off site for visiting clinics, Regional Training Adviser, SHA, College role etc) the time required to perform the role must be assessed and the implication of taking up the role discussed with the Clinical Director prior to the individual confirming acceptance of the role. This is to allow this type of leave to be discussed and agreed as part of the Job Planning process. To ensure equity, Clinical Directors should ensure that opportunities for participation in such approved external activity are rotated throughout the consultant body as appropriate.

The study leave budget will not normally be rolled-over to subsequent years.

Funding and Support

The total discretionary maximum allowance for each Consultant is £850 per annum pro rata.

Per capita budget levels for study/professional leave will be reviewed annually to ensure appropriate levels are maintained and individuals should be given equal access to the available funding. Individuals should be given equal access to the available funding; however, it is recognised that newly appointed staff may require

more support during their first three years in post than experienced staff. All study/professional leave should be aligned to Trust objectives and personal development.

For study leave, the full costs of approved course/conference fees will be funded up to the maximum allocation. Study leave costs may be granted for course/conference fees when attendance is during an individual's own time.

Travel expenses are reimbursed at the level of standard class return rail fare or the appropriate Trust mileage allowance for car drivers (public transport rates). Travel by taxi should be used only where other travel methods are not possible.

Travel expenses other than rail or car mileage may also be reimbursed where this is proven to be a cheaper alternative i.e. air travel in economy class. Upgrades on rail and airline services may be paid for by the individual with reimbursement by the Trust of the lowest economy fare available at the time of booking, provided evidence is supplied.

No funding will be given unless receipts are submitted with expense claim forms and submitted within one month of the leave.

It is expected that expenses, travel, and accommodation will be arranged as economically as possible and taking advantage of any discounts available for late/early/internet booking. Accommodation should be no more than a UK equivalent 3 to 4-star hotel.

Individuals may obtain external sponsorship for education and development activity. Where sponsorship is gained this should be indicated on the leave form, noting the amount of financial support obtained. Individuals should ensure that no conflict of interest will occur with such sponsorship and it should be discussed in the appraisal process and registered in the Trust's 'Register of Business Interests'.

Roles and Responsibilities

The Individual For any study/professional leave granted the individual is expected to:

- Ensure leave contributes to their PDP / Job Plan
- Attend all sessions for which study leave have been granted
- Inform Clinical Director if they have been unable to attend any sessions
- Pay back funding provided by the Trust, if the individual withdraws from or does not attend a funded course/conference without justifiable reason or

leaves soon after completion. This will be agreed between the individual and the Clinical Director.

- Ensure cover arrangements are in place for period of absence in conjunction with line manager.
- Share information / training with colleagues / wider organisation. This may be done informally through the team / department the individual works in, or the individual may be expected to share his/her learning more widely.

Clinical Director

For any education and development leave granted the Clinical Director is expected to:

- Ensure leave contributes to individual's PDP / Job Plan
- Ensure staff attend mandatory training through the appraisal process
- Plan study leave, consider implications on service needs
- Ensure an equitable and consistent approach is used when considering leave, using the framework contained in this procedure as a guide
- Consider the number and frequency of applications from one individual
- Consider the value of more than one person attending a particular event
- Approve applications for only one financial year at a time
- Keep a record of all leave/informing Rota Co-ordinators
- Enable staff to share and put into practice the skills they learn

Appeals

Appeals against a refusal to grant professional/study leave should, wherever possible, be resolved at a local level by discussion with the Clinical Director and if unresolved referred to the Chief of Division.

Trainee Doctors

Trainee doctors study leave is governed by Health Education East Midlands (HEEM) and advice from the Medical Staffing and Postgraduate Education Departments should be sought with regard to current procedures for such leave.

7. Special Leave

Special Leave Guidelines

In addition to the situations covered by the Trust's Special Leave Guidelines (for compassionate, carer, adoption and paternity leave – available on the Trust Intranet), it is sometimes necessary for Medical and Dental staff to take leave for

external work-related purposes, for which the Trust will agree to continue payment of salary and reimburse expenses. This is not an entitlement and individual circumstances will be taken into consideration and balanced against the needs of the service.

For Consultant staff external work can fall into the following categories:

Category 1 –

This is work which is reasonably incidental to a medical or dental staff member's duties under their contract of employment with the Trust and associated job description and job plan. Charges may not be made for such work. Category 1 work may include attendance at Court to give factual evidence on his/her own behalf or on behalf of the Trust.

Category 2 –

This relates mainly to all other work, examinations and reports which are not part of, or reasonably incidental to, a medical or dental staff member's normal contracted duties.

Whilst the Trust may grant leave with payment of salary and/or expenses in Category 1 cases, absences in respect of Category 2 work will not normally attract such payments unless taken during annual leave periods when payment of salary will continue.

Discretionary time off will not be agreed where this would be detrimental to service provision, teaching or the fulfilment of a consultant's fixed commitments, unless prior approval is given for alternative arrangements to be made. In the exceptional event that consultants have external commitments which require regular absence from a fixed commitment, agreement should be reached with the Clinical Director on how the commitment will be covered and/or whether the commitment may be rescheduled. This clause does not apply to consultants on the new contract who are undertaking external duties as part of their agreed job plan; these consultants will not be considered as being on leave.

8. Sickness Absence

Policies

Medical and dental staff are subject to the provisions of the Trust's Maintaining High Professional Standards Policy and the sickness absence reporting procedure detailed in the Trust Managing Attendance and Sickness Absence Policy, available on the Trust Intranet.

Allowances

Staff absent from duty owing to illness (including injury or other disability) shall, subject to the provisions of the terms and conditions of service, be entitled to receive an allowance in accordance with the following:

During the first year of service	One months full pay and two months half pay (after completing four months service)
During the second year of service	Two months full pay and two months half pay
During the third year of service	Four months full pay and four months half pay
During the fourth and fifth year of service	Five months full pay and five months half pay
After completing five years of service	Six months full pay and six months half

To aid rehabilitation the Trust has discretion to allow a doctor/dentist to return to work on reduced hours or to be encouraged to work from home without loss of pay to aid rehabilitation. Any such arrangements need to be consistent with statutory sick pay rules.

Calculation of Allowances

The rate of allowance shall be ascertained by deducting from the period any previous absences aggregated during the twelve months immediately preceding the first day of absence. In aggregating the periods of absence, no account shall be taken of:

- Unpaid sick leave; or
- injuries or diseases sustained to members of staff in the actual discharge of their duties through no fault of their own; or
- injury resulting from a crime of violence not sustained on duty but connected with or arising from the consultant's employment or profession, where the injury has been the subject of payment by the Criminal Injuries Compensation Authority; or

The Trust may at its discretion also take no account of the whole or part of the periods of absence due to injury (not on duty) resulting from a crime of violence not arising from or connected with the consultant's employment or profession.

Previous Qualifying Service

For the purpose of ascertaining the appropriate allowance of paid sick leave, all periods of service, without any break of twelve months or more (unless undertaking voluntary service), with a National Health Service employer shall be aggregated.

Limitation of Allowance When Insurance or Other Benefits are Payable 8

The sick pay paid when added to any statutory sickness, injuries or compensation benefits, including any allowances for adult or child dependants, must not exceed full pay.

Submission of Doctor's Statements of Fitness for Work

A staff member who is incapable of doing his or her normal work because of illness shall immediately notify the Trust according to departmental procedures. Any absence of more than seven days shall be certified by a doctor other than the sick doctor themselves. Statements of fitness for work must be submitted to the Clinical Director. These will normally be for a limited period and subsequent statements should be provided to cover the whole absence.

Accident Due to Sport or Negligence

An allowance shall not normally be paid in a case of accident due to active participation in sport as a profession, or in a case in which contributory negligence is proved, unless the Trust decides otherwise.

Injury Sustained on Duty

An absence due to injury sustained in the actual discharge of duty, for which the doctor/dentist was not liable, shall not be recorded for the purposes of these provisions.

Medical Examination

The Trust may at any time require a doctor/dentist who is unable to perform his or her duties as a consequence of illness to submit to an examination by a medical practitioner nominated by the Trust. Any expense incurred in connection with such an examination shall be met by the Trust.

Termination of Employment

After investigation, consultation and consideration of other alternative posts, and where there is no reasonable prospect of the doctor/dentist returning to work, the Trust will have the option to terminate employment before the employee has reached the end of the contractual period of sick leave, provided that the doctor/dentist will receive his or her entitlements.

9. Records

All annual leave will be recorded on the Trust Electronic Rostering system and Electronic Staff Record (ESR) for monitoring and reporting.

10. Training

There is an expectation that all employees of the Trust are responsible for ensuring their own individual awareness of this policy. However briefing sessions for managers and staff will be made available as necessary.

11. Monitoring Compliance with the Document

Monitoring Compliance					
Process to be used for monitoring e.g. audit	Responsible individual / committee for carrying out monitoring	Frequency of monitoring	Responsible individual / committee for review the results	Responsible individual / committee for developing action plan	Responsible individual / committee for monitoring action plan
Audit	Head of People Planning	Quarterly	People Committee	Head of People Planning	People Committee

12. References

The Medical and Dental Terms and Conditions of Service
The Consultant Contract Terms and Conditions of Service 2003
British Medical Association
Trust Special Leave Policy (A.3.3)
Trust Leave to in Relation to Starting a Family Policy (UHN-PO-HR10)
Trust Employment Break Scheme
Trust Managing Attendance and Sickness Absence Policy (B.4)
Maintaining High Professional Standards in the NHS (B.1.1)

Appendix A

Guidance on the booking of annual leave

- Annual leave entitlements are recorded in eRostering by Medical HR and requests managed via eRostering. Separate guidance/Quick reference Guides on how to view and approve/reject annual leave requests are available from the Workforce Team (HR Directorate)
- Requests for annual leave should be submitted no later than 6 weeks prior to the intended leave date using Employee On Line and will be approved/rejected electronically. Departmental protocols/Standard Operating Procedures in relation to thresholds for annual leave should be clearly communicated. Where any ambiguity lies between the departmental policy and this policy it is this policy that will take precedence.
- Consultants or designated deputy must approve all annual leave and ensure that sufficient staff remain on duty to cover the service safely at all times.
- No travel arrangements or holiday bookings can be made until the request has been approved and approval notification has been received via Employee On Line. Any costs incurred before a period of leave is formally sanctioned at the risk of the member of staff.
- Annual leave for new starters will only be honoured if discussed at interview/pre Appointment and appropriate supporting evidence provided.
- Half term weeks and school holidays pose additional problems and staff with children should not assume they have an automatic right to leave at these times. Discussions should be encouraged between those requesting half-term so that annual leave can be shared equally amongst them.
- Normally, staff should have taken 50% of leave by the end of September and have no more than 25% remaining in the final three months of the financial year.
- Annual leave must be booked or cancelled before a roster is approved.
- Consultants – or their deputy - must respond to notification requests for annual leave within 2 days of its receipt. (A response can include a timescale for responding rather than agreement or rejection).

Appendix B

NON TRAINEE STUDY/PROFESSIONAL LEAVE REQUEST (page 1)

Name	
Grade	
Specialty	

Leave Request	
Title of Event/Purpose (Attach details)	
Venue	
From	
To	
Total Days	
Are any other consultants on leave within your service on the requested dates above?	
Cover arrangements for Outpatients/Theatres, etc?	
Other cover arrangements	

Authorisation	Requested within 6 weeks and doctor compliant with mandatory training
Applicant signature	
Date	
Clinical Director signature	
Date	
Chief of Division/Divisional Director Signature	
Date	

NON TRAINEE STUDY/PROFESSIONAL LEAVE REQUEST (page 2)

EXPENSE CLAIM

Expenses Receipts must be attached	
Course/Conference Fees	£
Accommodation Costs Max £105 per night	£
Travel by Car 24p per mile	£
Other Travel Costs Details	£
Other Costs Details	£
Total Claim	

Sponsorship/Other Funding Please give details of any contribution towards fees and other expenses	
Details of funding organisation	
Total Funding	£
Has this been declared as a potential conflict of interest / Is this not a conflict of interest?	

Authorisation	
Applicant signature	
Date	
Clinical Director signature	
Date	
Chief of Division/Divisional Director Signature	
Date	

Appendix C

MEDICAL STAFF APPLICATION FOR SABBATICAL/CAREER BREAK

PART 1

Name	
Grade	
Specialty	

Leave Request	
Purpose of Leave	
From	
To	

Request	I understand that should my application be approved that I will be expected to adhere to the provisions of the Employment Break Policy
Applicant Signature	
Date	

PART 2

For Clinical Director to complete

I have considered this application for a career break considering service needs and the individual need and make the following recommendation

Approved / Not Approved	
Reasons for Decision	

Authorisation	I understand that should this application be approved that it will subject to the provisions of the Employment Break Policy
Clinical Director Signature	
Date	
Chief of Division / Divisional Director Signature	
Date	

DMG / Medical Director Decision	Approved / Not Approved
Date	